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A Proven, Provider-First Behavioral Health Platform

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Redefining The Behavioral Health Infrastructure

What We Are

An end-to-end platform combining tech and services to deliver a full-spectrum behavioral health ecosystem that is scalable across industries.

Why We'll Win

Clinician-built and market-tested, our platform addresses pain points for both providers and customers. With strong demand, loyal adoption, and no reliance on outside capital, we've proven the model and are ready to scale.

Traction

- $15–20M projected 2025 revenue
- $4-6M projected 2025 EBITDA (bootstrapped)
- 3,000+ VA evaluations/month
- Regional scaling of PI Division
- 120+ active providers

Opportunity

- A $4B+ underserved vertical market
- Sticky, defensible model
- Clear exit paths via spin-outs, M&A, or IPO

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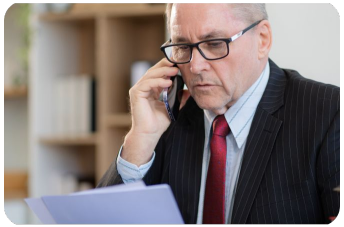
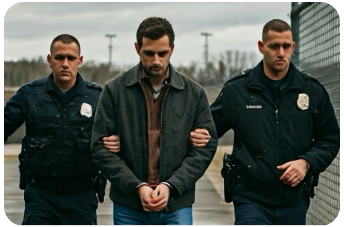
Behavioral Health Is Breaking Down Where It Matters Most

A veteran waits weeks for a mental health evaluation.



A personal injury plaintiff delays legal closure due to missed appointments.



A parolee returns to prison, not for crime, but for lack of behavioral support.

Different settings.
Same broken system.

The result?

- Missed care
- Lost revenue
- Burnout
- Delays
- Lawsuits
- Re-incarceration

WHY NOW

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A Converging Wave of Demand, Policy, and Risk Is Driving Market Change

Americans' Reported Mental Health at New Low; More Seek Help

BY MEGAN BRENAN

A growing psychiatrist shortage and an enormous demand for mental health services

The U.S. had too few psychiatrists even before COVID-19 increased rates of anxiety and depression. From partnering with primary care providers to innovative digital tools, leaders are stepping up with a range of creative solutions.

Demand for mental health is surging from 13% of Americans visiting a specialist in 2004 to 23% in 2022.

Rep. Morgan McGarvey, D-Ky., has reintroduced his Board of Veterans' Appeals Attorney Retention and Backlog Reduction Act to try help reduce the veterans claims appeal backlog at the VA. TOM WILLIAMS / GETTY IMAGES

Pay & Benefits

Efforts to boost VA recruitment and promote BOP mental health get another push from House members

Government agencies are being mandated to improve care access.

CIVHC

New Telehealth Analysis Shows Sustained Demand for Mental Health Services

The Center for Improving Value in Health Care (CIVHC) recently released its sixth update to the Telehealth Services Utilization Analysis, which now includes data from 2019 through 2023. The COVID-19 pandemic accelerated the adoption of telehealth services, reshaping health care delivery in Colorado. Using claims data from the Colorado All Payer Claims Database (CO APCD), the Telehealth Services Analysis provides critical insights into this

COVID catalyzed long-term demand for tele-behavioral care.

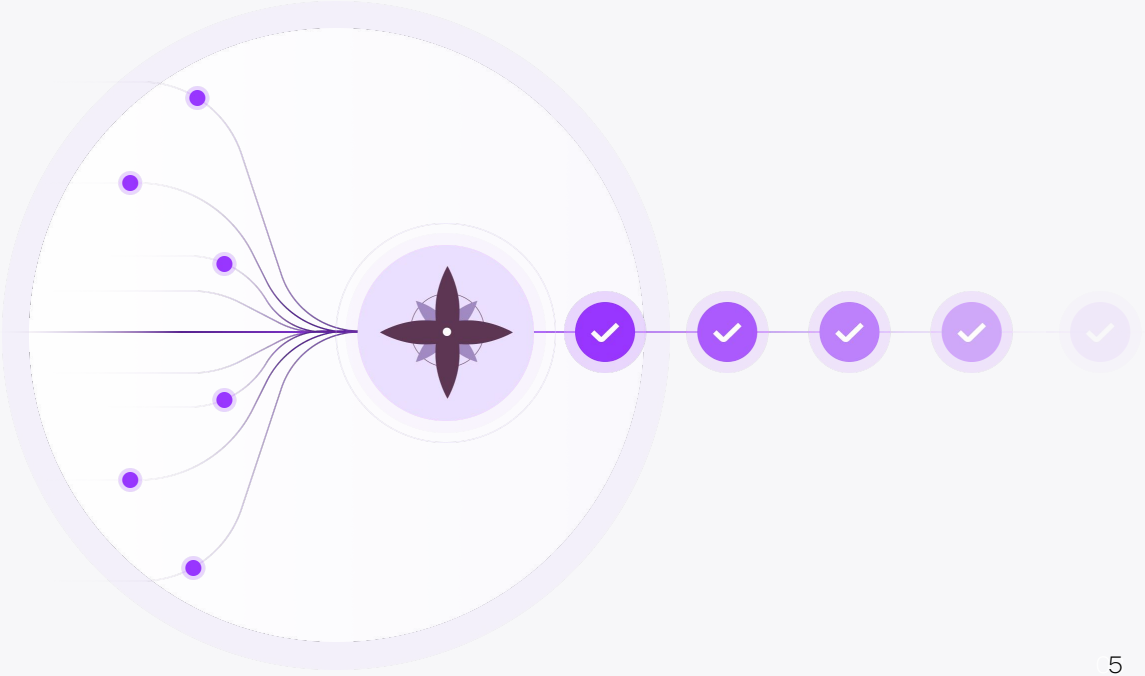
Neurology Practice Settles with HHS OCR for $25K

In a continued push to hold healthcare providers accountable for cybersecurity preparedness, the U.S. Department of Health and Human Services (HHS) Office for Civil Rights (OCR) has announced a settlement with *Comprehensive Neurology, PC*, a small neurology practice based in New York. The settlement resolves an OCR investigation following a ransomware

Rising fines for compliance failures are pushing providers to seek integrated solutions.

Today, care teams across healthcare, legal, and government systems rely on spreadsheets, faxed forms, and siloed EHRs to coordinate behavioral health services – creating delays, inefficiencies, and compliance risks.

With EmpowerMH, patients are onboarded, evaluated, and supported — all through a single platform designed around their needs and the providers delivering their care.



SOLUTION

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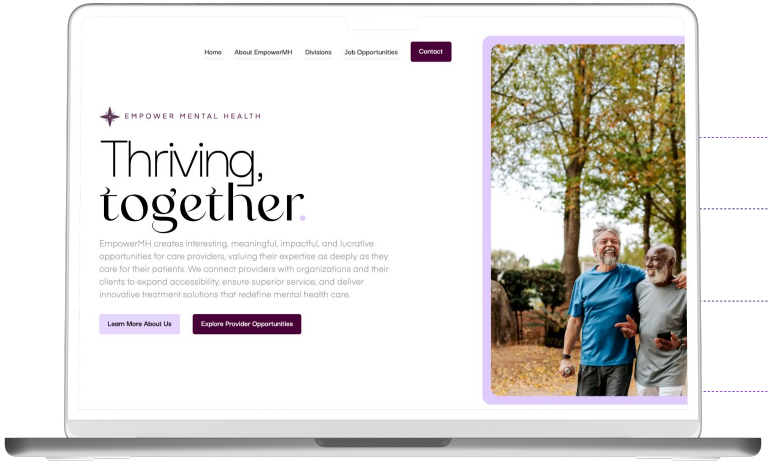
A Clinician-Built Platform That Works Because It Was Built Inside the System

A behavioral health infrastructure built by clinicians, for the sectors where outcomes matter most:

- Veterans' care
- Legal settlements
- Healthcare compliance
- Correctional reentry



We manage everything – from intake to outcomes:

- End-to-end workflows
- Clinical + operational + tech services
- Full-spectrum evaluations and counseling
- Provider-first design that reduces burnout

Example Use Case:

A parole officer logs into EmpowerMH to schedule a reentry evaluation, monitor a client's progress, and generate a compliance report. All on the same platform used by a VA counselor conducting disability evaluations and a PI attorney coordinating psychological assessments.

MARKET OPPORTUNITY

Large, Reachable, and Rapidly Growing Market

$9-18B

- 10–20% of U.S. behavioral health service spend
- Capture via coordination, outcomes tracking, and infrastructure

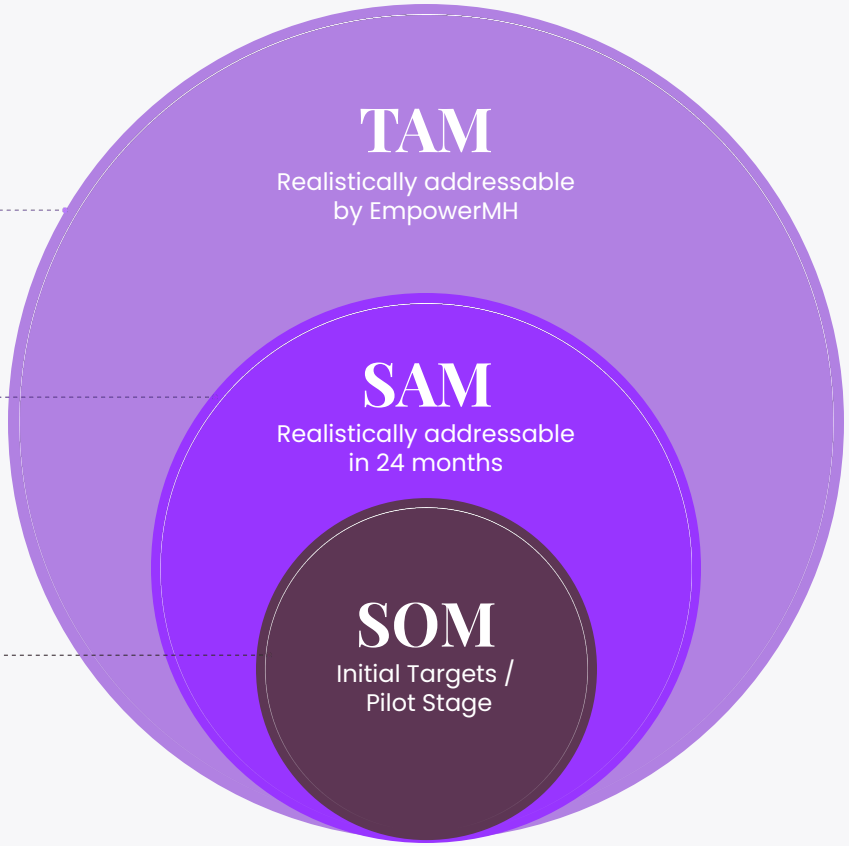
$4.2-4.8B

- VA contracts (15% capture): ~$2.6B
- Corrections (10%): ~$700M
- PI Firms (20%): ~$1.2B
- Focus: Coordination, staffing, oversight, and SaaS

$75-150M

Anchored in:

- 3,000+ monthly VA evaluations
- 2025 projected: $15–20M revenue, $4-5M EBITDA
- Initial PI casework already generating ARR



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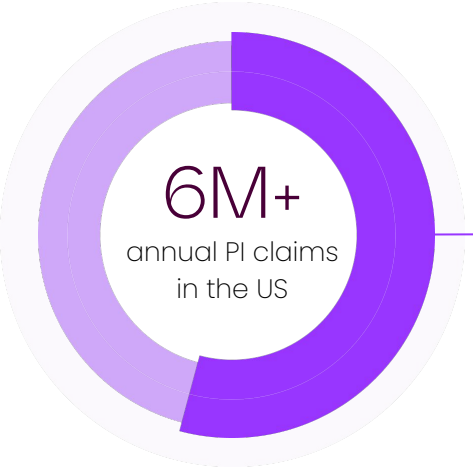
The U.S. PI Industry Is Primed for Behavioral Health Services

Millions of claimants face untreated trauma and cognitive issues, delaying settlements and reducing payouts. EmpowerMH recovers this lost value through integrated care.



3.5M
claimants need mental health care

Top issues:
- Trauma
- Anxiety
- TBI
- PTSD

$7,000
average per case

$23B+
total annual market

Highest opportunity:
- Car accidents ($10.5B)
- Dog bites ($6.3B)
- Slip & fall ($4.2B)

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Behavioral Health is Overlooked in the Medical System

43%

psychiatric outpatient non-adherence driving higher hospitalizations/ER use

$60B

excessive budget losses attributable to mental health

$13.7B/year

incremental spend for dialysis patients lacking BH support

$100-$300B/year

in avoidable costs driven by non-compliance issues

Zero integrated solutions

no existing EHR + billing + clinical workflows solutions addressing health care admin needs

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Proven Product-Market Fit with a Scalable Growth Path

2021

- $250K+ ARR
- First contracts in VA disability evaluation space

2022

- ~$500K+ ARR
- 75% increase in people served (total of 1,750)
- Early revenue from TBI/neuropsych evaluations and counseling

2023

- $1.6M ARR
- 35 onboarded providers from 7 in 2022
- VA evaluations 2,000+/mo
- Cross-vertical expansion: Legal, Healthcare, Government (federal contracts across US)

2024

- $7M+ ARR
- VA evaluations 3,000+/mo
- 60+ active providers
- LLCs launched for vertical- specific operations
- Entered the medical legal space
- Launch of PI (Personal Injury) service line
- Platform development start

2025

- Projected revenue $15-20M ($4-6M EBITDA)
- 120+ providers, projected 150+ by EOY
- Projected 4,000+/mo veterans served by EOY
- Increased Partnerships with federal contracts
- Corrections pilot development (HOPE reentry program)
- PI partnerships EOY projected 50+
- Tech platform implementation
  - EHR
  - Billing
  - Case management

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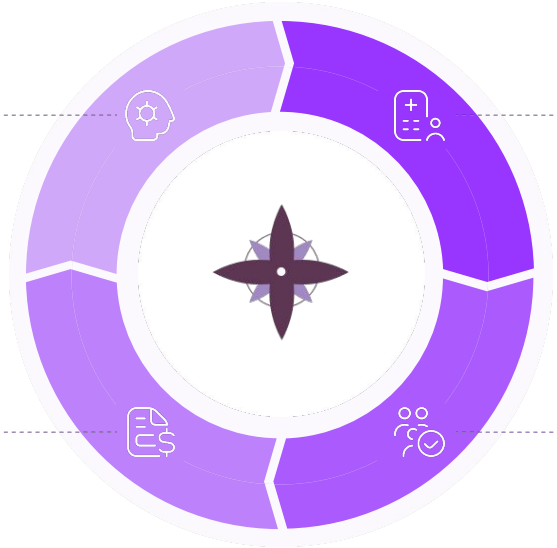
Designed with Clinicians. Defended by Execution.

Behavioral health demand isn't the challenge — building a loyal, high-performing provider network is. That's what makes EmpowerMH defensible.

Led by psychologists, not outsourced developers

Built by Mayo-trained medical and tech leaders



Integrated across EMR, billing, evals, reentry, compliance

Iterated daily with frontline staff input

The result?

- Low provider churn
- Faster onboarding
- Higher accuracy
- Sticky, defensible contracts

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Multiple Revenue Streams with Built-In Scalability

Service Revenue

- VA evals
- PI evals & counseling
  - Soon: Corrections/ HOPE contracts
  - Soon: Forensic services such as immigration evals etc.

Provider Subscriptions
(Platform/SaaS)

- Recurring revenue via monthly/annual subscriptions for tools:
  - EMR
  - Telehealth tools
  - AI assistant
  - Billing automation
  - Referrals

Separate LLCs

- Targeted spin-offs and streamlined M&A opportunities for verticals

Insurance Partnerships

- Direct payer contracts deliver 20–30% base margins (for PI over 100% margins)

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Plan for Scaling Across Verticals

PHASE 1
PI, VA, corrections (now)

0-18 months

- Expand contracts in Healthcare, Legal, Government
- Secure additional contracts: VA, federal, and direct behavioral health service with correctional systems
- Develop new PI law firm partnerships
- Scale HOPE reentry pilot program

PHASE 2
Managed care + insurers

18-36 months

- Partner with auto insurers and managed care organizations
- Partner with healthcare insurance to reduce noncompliance issues
- Grow referral pipelines and joint programming

PHASE 3
SaaS and white-label expansion

24-48 months

- Launch white-labeled SaaS modules to third-party provider networks
- Onboard new enterprise channels (e.g., large health systems)

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Financial Projections

Year	ARR	EBITDA	Key Growth Drivers
2024	$7M	$2–3M	Bootstrapped growth; VA contracts & early PI traction
2025	$15-20M	$4–6M	Expansion in VA, PI & corrections; 200 PI cases/month
2026	$45M	$20-25M	Scaled PI operations; onboarding SaaS users; managed care pilots
2027	$120M	$45–55M	SaaS expansion; platform licensing; federal growth
2028	$200M	$80–100M	Full-scale rollout across 3 verticals

Estimated

Key Assumptions

- VA Services: Stable growth with 3,000+ evaluations/month
- PI Vertical: $10K per case, scaling to 200+ cases/month by Q4 2025
- SaaS & Subscriptions: 80%+ margins on platform licensing and provider tools
- Corrections & Healthcare: Multi-state expansion and managed care onboarding drive 30%+ margins
- 2025 funding round to fuel vertical scaling and recurring revenue growth from SaaS and service lines

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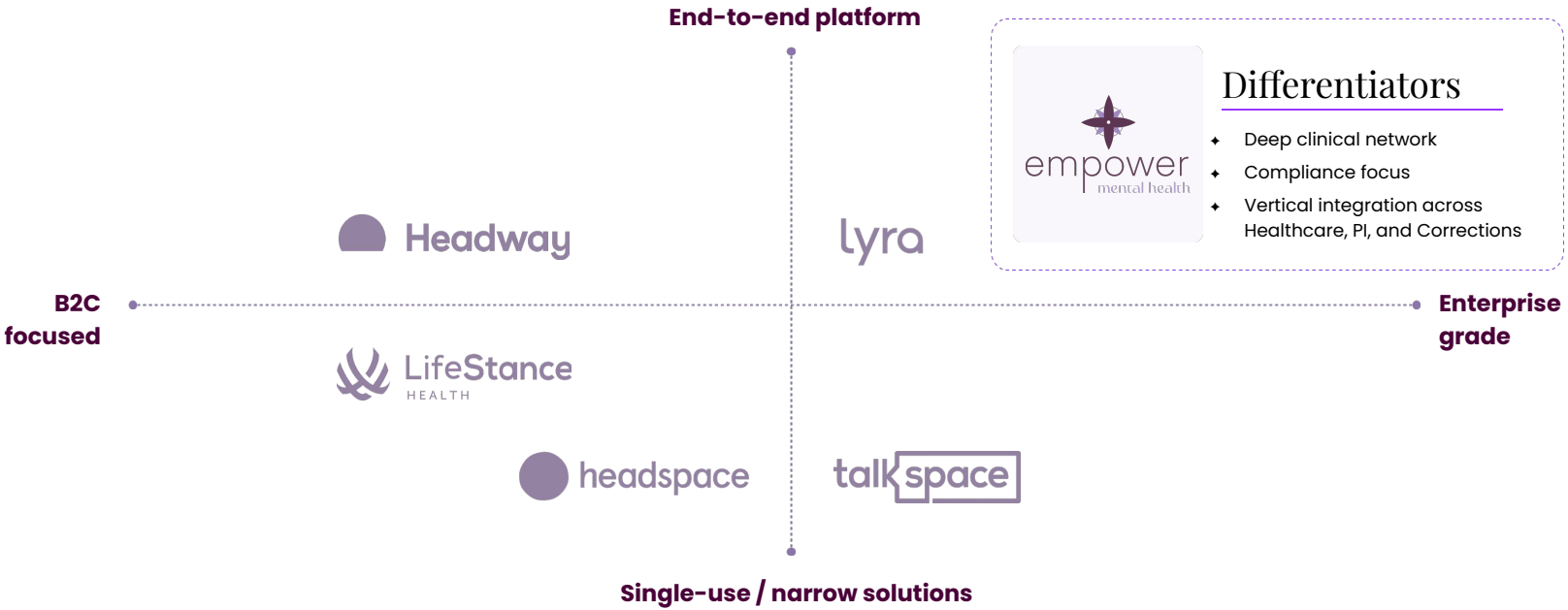
Competitive Comparison



While others offer point solutions, EmpowerMH delivers a highly scalable infrastructure.

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Domain Expertise Across Clinical, Technical, and Operational Fronts

Yisroel Loeb, PhD

CEO

Licensed clinical psychologist with deep experience in VA disability evaluations and corrections mental health systems.

Dr. Sherry Craft, Ph.D

CCO

PhD in Counseling Psychology with 15+ yrs in forensic and corrections psychology.

Samuel Reinstein

CFO

Started at Prudential and Munich Re, later founded a financial health company.

Ben Blech, MD

CTO

Mayo Clinic trained stroke neurologist who also specializes in EHR optimization, workflow automation, AI analytics, and HIPAA-compliant data integration.

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We're Raising to Build the Scalable Behavioral Health Infrastructure

We've already proven the model:

- $15M revenue
- 3,000+ VA evaluations per month
- Legal and corrections pipelines scaling now

Now we're raising

$$

To give us a 30-month runway to:

- Scale our tech platform across sectors
- Expand H.O.P.E. reentry and compliance programs
- Launch SaaS licensing for broader reach

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Partner with us

to transform the future of behavioral healthcare
and be part of a high-impact investment opportunity.

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Appendix

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Behavioral Health Gaps Weaken Legal Outcomes

35%

no-show rate for standalone EI/TBI evaluations, increasing amount of delayed settlements

Data fragmentation

Attorneys juggle 3–4 unconnected platforms for intake, testing, and documentation

25%

longer case durations when psych evals are outsourced or sparse

No unified "nexus statement"

multiple vendors needed for full scope of services

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The Burning Demand for Professional Clinical Infrastructure

$8B

spent on probation/parole violations (2021); most lacking BH support

$117K-$150K

recidivism cost per event

600K

individuals released annually

67%

rearrested within 3 years

Zero scalable platforms

Heavy reliance on manual workflows increases risk of human error and operational costs